AC-03 Adult Cardiac Arrest

MEDICAL-DIRECTOR APPROVED APP CONTENT | AGENCY RELEASE PENDING | Reconciled October 10, 2026

Scope: Use for patients age 16 years or older who are unresponsive, have absent or abnormal breathing, and have no definite pulse within 10 seconds. Honor valid DNR/MOST and obvious-death criteria.

Immediate coordinated resuscitation

1. Begin CPR	Compressions 100-120/min, depth 2-2.4 in, full recoil, and minimal pauses. Change compressor every 2 minutes or sooner if fatigued. Target chest-compression fraction above 80%.
2. Attach AED / monitor	Analyze rhythm immediately. Resume CPR immediately after every analysis or shock. Check a pulse only with an organized rhythm and limit the check to 10 seconds.
3. Rhythm pathway	VF/pulseless VT: open AC-09. Asystole/PEA: open AC-01. Repeat rhythm analysis every 2 minutes.
4. Access + medication	One rapid IV attempt, then IO if unsuccessful or delayed. Epinephrine 1 mg IV/IO every 3-5 minutes: as soon as possible for asystole/PEA and after the second shock for VF/pulseless VT.
5. Airway + ventilation	Use two-person BVM first and 30:2 before an advanced airway. Do not interrupt compressions for airway placement. After an advanced airway, give 1 breath every 6 seconds with continuous compressions and waveform capnography.
6. Reassess	Treat reversible causes, confirm mechanical pulse when an organized rhythm appears, move to AC-10 after ROSC, or consider AC-12 when resuscitation remains unsuccessful.

Shockable-rhythm medication and defibrillation

Therapy	Approved dose / use
Defibrillation	For VF/pulseless VT, give an initial 200 J biphasic shock. Verify the manufacturer-recommended setting; use the same or higher energy for subsequent shocks. Resume CPR immediately after every shock.
Epinephrine	1 mg IV/IO every 3-5 minutes. Give after the second shock for VF/pulseless VT; give as soon as possible for asystole/PEA.
Amiodarone	300 mg IV/IO after the third shock for refractory VF/pulseless VT; may repeat 150 mg IV/IO once.
Lidocaine	Alternative to amiodarone after the third shock: 1-1.5 mg/kg IV/IO initially; then 0.5-0.75 mg/kg every 5-10 minutes; maximum total 3 mg/kg. Do not combine with amiodarone without Medical Control.

Cause-directed care: Evaluate and treat the Hs and Ts. Do not routinely give calcium, sodium bicarbonate, magnesium, fluid, antidotes, or other cause-specific therapy without an appropriate suspected or confirmed cause.

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SPECIAL CIRCUMSTANCES, SAFEGUARDS, AND DOCUMENTATION

Special circumstances and linked pathways

Pregnancy	Begin standard resuscitation. When the fundus is at or above the umbilicus, use continuous left uterine displacement, notify Medical Control early, and plan an obstetric-capable destination when clinically appropriate. Use AO-03 for related obstetric care.
Traumatic arrest	Use TB-10 because hemorrhage control, oxygenation, bilateral chest decompression, and transport decisions differ from primary medical arrest.
Opioid / toxicologic	Naloxone or an antidote may be considered only when it does not delay CPR, defibrillation, airway support, or standard cardiac-arrest medications. Use TE-07.
Drowning / hypothermia	Use TE-03 or TE-05 because oxygenation, pulse assessment, medication timing, resuscitation duration, and transport priorities may differ.
Mechanical circulatory device	Use AC-14 or the applicable device pathway. A continuous-flow device may not produce a palpable pulse; assess perfusion and device function before starting compressions.

Safety checks

• High-quality CPR and early defibrillation for a shockable rhythm take priority over vascular access, medication preparation, airway placement, and scene movement.

• Do not routinely transport during active CPR. Favor on-scene resuscitation unless a defined special circumstance supports transport or ROSC is sustained.

• Do not use push-dose epinephrine, routine head-up CPR, double-sequential defibrillation, vector-change defibrillation, or point-of-care ultrasound under this protocol.

• A sudden sustained EtCO2 increase may suggest ROSC; do not use a single EtCO2 value or any single finding alone to terminate resuscitation.

Documentation and handoff

• Document downtime and witnessed status when known, bystander CPR/AED use, initial and subsequent rhythms, compression-quality interventions, shocks/energy, medication times and doses, airway/capnography findings, suspected cause, ROSC, and disposition.

• For ROSC, transition immediately to AC-10 and notify the receiving facility early. For persistent arrest, apply AC-12 and Medical Control requirements when appropriate.

• This document matches approved app content reconciled October 10, 2026. Final agency release remains subject to Claiborne County EMS release governance.